effect of deep sclerectomy is independent of external filtration (subconjunctival bleb), rarely occurring in only 5% of patients, and cite an article on viscocanalostomy as a reference.[2] Again, we feel it necessary to distinguish viscocanalostomy from deep sclerectomy with or without an implant. Although both are nonpenetrating surgeries, bleb formation after viscocanalostomy is rare due to tight closure of the superficial flap[2] (which was not performed by the authors in their viscocanalostomies in this report), whereas low, diffuse blebs are commonly found in deep sclerectomy, where the superficial scleral sutures are loosely tied.[4]

The authors should be commended for attempting a technically demanding procedure in these very challenging cases. However, it must be stated that this study was not able to appropriately measure the effectiveness of deep sclerectomy in refractory congenital glaucoma, the stated objective of the article. This study can only conclude that performing deep sclerectomy is technically difficult in these cases. It may be a fact that the authors had the intention of doing deep sclerectomy, but as the saying goes, some roads are paved with good intentions.

We do feel that the role of nonpenetrating glaucoma surgery in congenital glaucoma is yet undefined. In our experience, we have found reasonable success with an improved safety profile using this approach, and we will be publishing our results soon.

Tarek Shaarawy, MD
*Giza, Egypt*

Iqbal Ike K. Ahmed, MD, FRCS(C)
*Toronto, Canada*

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## Author reply

Dear Editor:
In their comment on our retrospective case series, Drs. Shaarawy and Ahmed state that our results contradict those of Tixier et al,[1] who published reasonable success rates after deep sclerectomy in patients with congenital glaucoma. In this context they may have overlooked that in our series all 10 eyes had a history of antiglaucomatous surgery with an average of 2.5 glaucoma interventions (Table 1), as indicated by the term "refractory congenital glaucoma." In the cited series of Tixier et al, most of the eyes had no previous surgery, with only 3 of 12 eyes having two interventions before deep sclerectomy. Thus, this comparison remains at least questionable.

Drs. Shaarawy and Ahmed recommended carefully distinguishing viscocanalostomy from deep sclerectomy due to different bleb rates. They indicated that bleb formation after viscocanalostomy is rare,[2] whereas it is commonly found after deep sclerectomy.[3] However, this substantially contradicts the recent review article[4] from their own study group stating that "50% of patients who underwent a viscocanalostomy had a subconjunctival filtering bleb." We feel that a watertight closure of a thin scleral flap in a buphthalmic eye is more likely a theoretical aim than practically feasible.

Deep sclerectomy is the essential surgical step of nonpenetrating techniques including viscocanalostomy. The first goal of these techniques is to unroof Schlemm's canal by the dissection of the deep scleral flap and thereby achieve a significant aqueous flow into the intrascleral space. In our series of refractory congenital glaucomas, in 60% of eyes this goal was not reached. However, a considerable rate of complications was observed.

Based on these results, we feel justified in drawing attention to the possible risks of nonpenetrating surgery in these rare cases of refractory congenital glaucoma with extremely thin sclera and pathological topography of Schlemm's canal.

Christoph Lüke, MD
Thomas S. Dietlein, MD
Philipp C. Jacobi, MD
Walter Konen, MD
Günter K. Krieglstein, MD
*Köln, Germany*

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Dear Editor:
We have a comment on the article on the risks profile of deep sclerectomy for treatment of refractory congenital glaucomas by Luke et al.

The pressure-lowering effect of deep sclerectomy is independent of external filtration only when the superficial scleral flap is tightly secured, as described in Stegmann's viscocanalostomy. External filtration is also absent when the scarring process is so important that the pressure-lowering effect of the surgery is minimal. In the article by Luke et al, only one of the eyes that underwent deep sclerectomy alone, with or without perforations, developed a filtering bleb. It was, moreover, the only one that achieved intraocular pressure (IOP) control. This is the reason why, to optimize all the possible outflow mechanisms past the trabeculodescemetic membrane, we tie our superficial flap very loosely, provided no visible perforation is present.

Of the 10 eyes in this series, only 2 had uneventful pure